



Corporate Office : HY PATHO LAB

CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028

7718962488

info@hyatholab.in

www.hyatholab.in

Toll Free No. : 18001030287

REPORT

PT Name : MR. RITIK WAGHELA

Ref By : DR. SELF

Reg No : HL3001832548 / HYMH/NALSPR/H7654

Barcode : HEN111259

INV : ABO Group & RH Type (Blood Group)

Age : 26 Year | Sex : Male

Registered on : 31-08-2026 07:33 PM

Received on : 31-08-2026 08:10 PM

Reported on : 31-08-2026 08:16 PM

SAMPLE COLLECTED AT :

GSDDPL



SAMPLE : EDTA Blood

ABO GROUP & RH TYPE (BLOOD GROUP)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
ABO Group Method: Erythrocyte Magnetized Technology	B	.	..
Rh Factor Method: .	POSITIVE	.	..

Clinical significance :

This ABO and Rh blood typing test identifies the presence of specific red cell antigens and antibodies to determine the ABO/Rh type.

Please correlate with clinical conditions

~~End of report~~

NRahate

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Pallavi

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)



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REPORT

PT Name : MR . MOHD ISRAFIL SHAIKH .

Age : 32 Year | Sex : Male

Ref By : DR. J. J. HOSPITAL .

Registered on : 31-08-2026 11:46 AM

Reg No : HL2901831865 / RAJ38

Received on : 31-08-2026 06:15 PM

Barcode : HEL001504

Reported on : 31-08-2026 10:58 PM

SAMPLE COLLECTED AT :

ARIBA DIAGNOSTIC CENTRE



INV : Absolute Eosinophil Count (AEC)

SAMPLE : EDTA Blood

ABSOLUTE EOSINOPHIL COUNT (AEC)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
AEC	560	/cmm	50 - 550

Method: Electrical Impedance

Reference Range

Peripheral Blood Eosinophilia	Counts/ μ L of Blood
Normal level of eosinophils	50-550
Eosinophilia	>500-1,500
Hypereosinophilia	>1,500 recorded on $\geq$ 2 determinations with minimum interval of 4 wks (unless immediate designation is required to initiate therapy because of eosinophil-related organ dysfunction)

USES:

Absolute eosinophil count is increased in allergic or atopic disease, infectious disorders (including parasites), medications, immunologic reactions, skin disorders, pulmonary syndromes, rheumatologic diseases, myeloproliferative neoplasms and secondary to other malignancies.

~~End of report~~

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REPORT

PT Name : YASHPAL SURYAWANSHI

Ref By : DR. SELF

Reg No : HL5001830758 / MH103/MH600

Barcode : HEN195136

INV : Activated Partial Thromboplastin Time (APTT/PTTK)

Age : 38 Year | Sex : Male

Registered on : 30-08-2026 12:31 AM

Received on : 30-08-2026 08:46 AM

Reported on : 30-08-2026 12:02 PM

SAMPLE COLLECTED AT :

SHRI RAM SAMARTH LAB



SAMPLE : Citrated Plasma

ACTIVATED PARTIAL THROMBOPLASTIN TIME (APTT/PTTK)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Patient Value Method: (Photo Optical Clot Detection)	30.2	sec	22 - 34
Control Value Method: Photo optical Clot Detection	28.0	sec	. - 28

NOTE-

Degree of prolongation of PTT / APTT is neither predictive of bleeding risk nor underlying diagnosis 2. Results should be clinically correlated 3. Test conducted on Citrated plasma

Comments

Partial Thromboplastin time (PTT / APTT) measures the proteins of the intrinsic coagulation pathway which consists of Factor XII, Prekallikrein, High molecular weight kininogen, Factors VIII, IX & XI. It also measures proteins of the common pathway namely factors II, V, X & Fibrinogen. PTT is prolonged when Factor VIII level is < 35-40% of normal and Factor XII & High molecular weight kininogen is < 10-15% of normal

Abnormal Partial Thromboplastin Time

Associated with bleeding: Defects of factors VIII, IX & XI · Not associated with bleeding: Defects of factor XII, Prekallikrein, High molecular weight kininogen & Lupus anticoagulants

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : KAMAL HEGADE
Ref By : DR. MANOJKUMAR WAGHMODE M.D.
Reg No : MEDICINE
Reg No : HL4601825373 / PUN104
Barcode : Hen183381
INV : Adenosine Deaminase (ADA) For Fluids

Age : 57 Year | Sex : Female
Registered on : 25-08-2026 02:19 AM
Received on : 25-08-2026 08:27 AM
Reported on : 25-08-2026 05:12 PM

SAMPLE COLLECTED AT :



SAMPLE : Body Fluid

ADENOSINE DEAMINASE (ADA) FOR FLUIDS

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
ADA FLUID Method: (ENZYMATIC)	11.8	IU/L	0.0 - 30.0

Clinical significance :

Adenosine deaminase (ADA) is a protein that is produced by cells throughout the body and is associated with the activation of lymphocytes, a type of white blood cell that plays a role in the immune response to infections. Conditions that trigger the immune system, such as an infection by *Mycobacterium tuberculosis*, the bacteria that causes tuberculosis (TB), may cause increased amounts of ADA to be produced in the areas where the bacteria are present. This test measures the amount of adenosine deaminase present in fluid in order to help diagnose a tuberculosis infection of the fluid.

fluid are membranes that cover the chest cavity and the outside of each lung. Small amounts of fluid are continuously produced to lubricate the movement of the lungs against these membranes and the membranes against each other during inhalation and exhalation. A variety of conditions and diseases, including infection, can cause inflammation of the fluid (pleurisy or pleuritis) and can lead to excessive pleural fluid accumulation (effusion).

Tuberculosis can spread into the lungs and pleurae, causing symptoms such as chest pain, chronic cough, and shortness of breath. Since these symptoms may also be seen with a variety of other conditions, it is important to determine the cause as rapidly as possible in order to properly treat the affected person. Detecting mycobacteria in pleural fluid can be difficult because there may be a large volume of fluid and very low numbers of bacteria present. Though the ADA test is not specific and does not replace the culture for diagnosing TB, it may be positive even when numbers of bacteria are very low and can be used as an adjunct test to help determine whether tuberculosis is the likely source of a person's symptoms.

Please correlate with clinical conditions

~~End of report~~

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REPORT

PT Name : MAST ASHIR SHAIKH

Age : 14 Year | Sex : Male

Ref By : DR. DR TALIB SURVE

Registered on : 29-08-2026 04:27 PM

Reg No : HL1501830432 / MH290

Received on : 29-08-2026 07:08 PM

Barcode : HEN154669

Reported on : 29-08-2026 08:35 PM

INV : Adenosine Deaminase (ADA) For Serum

SAMPLE COLLECTED AT :

STAR PATHOLOGY LABORATORY(SHILPHATA)



SAMPLE : Serum

ADENOSINE DEAMINASE (ADA) FOR SERUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
ADA SERUM Method: (ENZYMATIC)	25.1	IU/L	0.0 - 43.00

NOTE-

Increased Serum Levels -

Viral hepatitis

Infectious mononucleosis

Typhoid fever

Cirrhosis of liver and certain malignant tumors

Tuberculosis

Clinical significance :

Adenosine deaminase (ADA) is a protein that is produced by cells throughout the body and is associated with the activation of lymphocytes, a type of white blood cell that plays a role in the immune response to infections. Conditions that trigger the immune system, such as an infection by *Mycobacterium tuberculosis*, the bacteria that causes tuberculosis (TB), may cause increased amounts of ADA to be produced in the areas where the bacteria are present. This test measures the amount of adenosine deaminase present in body in order to help diagnose a tuberculosis infection of the body.

Tuberculosis can spread into the lungs and pleurae, causing symptoms such as chest pain, chronic cough, and shortness of breath. Since these symptoms may also be seen with a variety of other conditions, it is important to determine the cause as rapidly as possible in order to properly treat the affected person. Detecting mycobacteria in pleural fluid can be difficult because there may be a large volume of fluid and very low numbers of bacteria present. Though the ADA test is not specific and does not replace the culture for diagnosing TB, it may be positive even when numbers of bacteria are very low and can be used as an adjunct test to help determine whether tuberculosis is the likely source of a person's symptoms

Please correlate with clinical conditions

~~End of report~~

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REPORT

PT Name : PARTH PATIL
Ref By : DR. SELF
Reg No : HL9601831704 / MH009
Barcode : HEN179168
INV : Albumin, Serum

Age : 17 Year | Sex : Male
Registered on : 31-08-2026 09:11 AM
Received on : 31-08-2026 11:04 AM
Reported on : 31-08-2026 12:30 PM

SAMPLE COLLECTED AT :



SAMPLE : Serum

ALBUMIN, SERUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Albumin, Serum Method: (PHOTOMETRY)	4.22	gm/dl	3.2 - 5.2

Comments :

An albumin blood test measures the amount of albumin in your blood. Albumin is a protein made by your liver. Albumin helps keep fluid in your bloodstream so it doesn't leak into other tissues. It is also carries various substances throughout your body, including hormones, vitamins, and enzymes. Low albumin levels can indicate a problem with your liver or kidneys.

Clinical significance :

- Albumin is a carbohydrate-free protein, which constitutes 55% to 65% of total plasma protein. It maintains oncotic plasma pressure, is involved in the transport and storage of a wide variety of ligands, and is a source of endogenous amino acids. Albumin binds and solubilizes various compounds, including bilirubin, calcium, long-chain fatty acids, toxic heavy metal ions, and numerous pharmaceuticals.

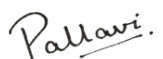
Hypoalbuminemia is caused by several factors: impaired synthesis due either to liver disease (primary) or due to diminished protein intake (secondary), increased catabolism as a result of tissue damage and inflammation, malabsorption of amino acids, and increased renal excretion (eg, nephrotic syndrome).

Please correlate with clinical conditions

~~End of report~~



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REPORT

PT Name : MR IMRAN PATEL

Ref By : DR. RAHUL T CHAKOR

Reg No : HL6001831923 / MH593

Barcode : HEN116971

INV : Alkaline Phosphatase (ALP)

Age : 46 Year | Sex : Male

Registered on : 31-08-2026 12:09 PM

Received on : 31-08-2026 01:28 PM

Reported on : 31-08-2026 03:28 PM

SAMPLE COLLECTED AT :



SAMPLE : Serum

ALKALINE PHOSPHATASE (ALP)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Alkaline Phosphatase Method: FS IFCC	72	U/L	53.00 - 128.00

Interpretation

Alkaline phosphatase in serum consists of 4 structural genotypes: the liver-bone-kidney type, the intestinal type, the placental type, and the variant from the germ cells. It occurs in osteoblasts, hepatocytes, leukocytes, the kidneys, spleen, placenta, prostate, and the small intestine. The liver-bone-kidney type is particularly important.

A rise in the alkaline phosphatase occurs with all forms of cholestasis, particularly with obstructive jaundice. It is also elevated in diseases of the skeletal system, such as Paget disease, hyperparathyroidism, rickets and osteomalacia, as well as with fractures and malignant tumors. A considerable rise in the alkaline phosphatase activity is sometimes seen in children and juveniles. It is caused by increased osteoblast activity following accelerated bone growth.

Please correlate with clinical conditions.

~~End of report~~

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REPORT

PT Name : SUCHI MARFATIA

Ref By : DR. SELF

Reg No : HL6601832626 / HYMH/KDV/H7275

Barcode : HEN172678

INV : Amylase (Serum)

Age : 58 Year | Sex : Female

Registered on : 02-09-2026 10:50 AM

Received on : 02-09-2026 05:14 PM

Reported on : 02-09-2026 08:57 PM

SAMPLE COLLECTED AT :



SAMPLE : Serum

AMYLASE (SERUM)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
AMYLASE	59.25	U/L	28 - 100

Method: PHOTOMETRY

Clinical significance :

Lipemic Sera (Hypertriglyceridemia) may contain inhibitors, Which falsely depress results. About 20% of patients with Acute Pancreatitis have abnormal lipids. Normal serum amylase may occur in Pancreatitis, Especially relapsing and chronic pancreatitis. Moderate increases may be reported in normal pregnancy.

Clinical Significance:

Causes of high Serum Amylase include Acute Pancreatitis, Pancreatic Pseudocyst, Pancreatic Ascites, Pancreatic Abscess, Neoplasm in or adjacent to Pancreas, Trauma to Pancreas, and common Duct Stones. Nonpancreatic Causes include inflammatory salivary lesions (Eg, Mumps), Perforated Peptic Ulcer, Intestinal Obstruction, Biliary Tract Disease, Peritonitis, Acute Appendicitis, Diabetic Ketoacidosis, and Extrapaneatic Carcinomas. Amylase levels more than 25-fold the upper limit of normal are often found when metastatic tumors produce Ectopic Amylase.

Please correlate with clinical conditions

~~End of report~~



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